statistics is private expenditure on health insurance. In 2016–17, this was estimated at Rs 0.27 lakh crore, or about 0.2 per cent. According to government estimates, total health expenditure (by the government and households), including capital expenditure by the government, was 3.6 per cent of GDP in 2016–17.

Health Situation Remains Grim

The Ministry of Health published a document titled ‘Situational Analysis—Backdrop to the National Health Policy-2017’ before NHP-2017 was adopted. The document candidly acknowledges the economic, financial and fiscal challenges in the health sector.

On fiscal capacity, the document states: ‘the fact however remains that inability to cover the entire spectrum of healthcare needs through increased public investments has led to a rise in out-of-pocket expenditure and consequent impoverishment.’ The fact that rising health costs are borne privately by poor Indians is acknowledged candidly: ‘Over 63 million persons are pushed to poverty every year due to healthcare costs. In 2011–12, the share of out-of-pocket expenditure on healthcare as a proportion of total household monthly per capita expenditure was 6.9 per cent in rural areas and 5.5 per cent in urban areas. This led to an increasing number of households facing catastrophic expenditures due to health costs (18 per cent of all households in 2011–12 compared to 15 per cent in 2004–05).’

It recognizes that the coverage of publicly funded health insurance programmes, the central Rashtriya Swasthya Bima Yojana (RSBY) programme and state-funded trust model programmes, did expand quite well but still remained inadequate: ‘The population coverage under these various schemes expanded from almost 55 million in 2003–04 to 370 million in 2014 (almost one-fourth of the population).’

On the private healthcare industry, the document grudgingly admits: ‘The private healthcare industry is complex. It includes insurance and